



Drug Management Policy

Stelara

Policy Number: 9.129

Version Number: R5

Version Effective Date: 3/1/2025

Impacted Products

- ☐ **All Products**
- ☒ NH Medicaid
- ☐ NH Medicare Advantage
- ☐ MA MassHealth ACO
- ☐ MA MassHealth MCO
- ☐ MA Clarity plans/Employer Choice Direct
- ☐ MA Senior Care Options

Benefit

- ☒ Pharmacy benefit
- ☒ Medical benefit

Note: Disclaimer and audit information is located at the end of this document.

Prior Authorization Policy

Products Affected:

- Stelara (ustekinumab) subcutaneous injection
- Stelara® (ustekinumab) IV (induction of therapy only)

The Plan may authorize coverage of the above products for members meeting the following criteria:

Covered Use	All medically accepted indications not otherwise excluded
Exclusion Criteria	Use in combination with other biologics
Required Medical Information	Moderate to severe Active Crohn's Disease (CD); AND 1. The member is 18 years of age or older; AND 2. The requested medication is prescribed by or in consultation with a gastroenterologist; AND 3. Documentation of ONE of the following: a. An inadequate response or adverse reaction to at least a three consecutive month trial of at least ONE immunomodulator (e.g. azathioprine, 6-mercaptopurine, methotrexate) ; OR b. An inadequate response or adverse reaction to at least a three consecutive month trial of one biologic DMARD that is FDA-approved for Crohn's disease; OR c. Supporting documentation (chart notes) of the member's Harvey Bradshaw Index score (prior to treatment with the requested agent) indicating moderately active (8-16) or severely active (17-100) disease; AND 4. Documentation of ONE of the following: a. An inadequate response or adverse response to at least a three consecutive month trial of a preferred adalimumab biosimilar product (adalimumab-aacf, adalimumab-adaz, adalimumab-adbm, adalimumab-fjkg, adalimumab-ryvk),, Rinvok, or infliximab (generic for Remicade); OR b. A contraindication to all of the following: adalimumab, infliximab (generic for Remicade), and Rinvok ; AND 5. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A); AND 6. If the request if for Stelara IV BOTH of the following:

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- a. The member will receive induction dosing with Stelara intravenous within two months of initiating therapy with Stelara subcutaneous; **AND**
- b. It will be used for induction therapy only.

Moderate to severe plaque psoriasis (PsO)

For the approval of the 45 mg syringe/vial:

- 1. The member is 6 years of age or older; **AND**
- 2. The requested medications prescribed by or in consultation with a dermatologist or rheumatologist; **AND**
- 3. ONE of the following:
 - a. Involvement of at least 3% of total body surface area; **OR**
 - b. Hands, feet, scalp, face, or genital area affected; **AND**
- 4. Documentation of ONE of the following:
 - a. An inadequate response or adverse reaction to at least a three consecutive month trial of any one of the following combinations (please note: these combinations DO NOT have to be used concurrently):
 - i. one topical agent plus one systemic agent; **OR**
 - ii. one topical agent plus one phototherapy; **OR**
 - iii. one systemic agent plus one phototherapy; **OR**
two systemic agents; **OR**
 - iv. A contraindication to all conventional therapies (topical agents, phototherapy, and systemic agents); **OR**
 - b. An inadequate response or adverse reaction to at least a three consecutive month trial of one biologic DMARD that is FDA-approved for plaque psoriasis; **AND**
- 5. Documentation of ONE of the following:
 - a. An inadequate response or adverse reaction to at least a three consecutive month trial of Enbrel, of a preferred adalimumab biosimilar product (adalimumab-aacf,

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adalimumab-adaz, adalimumab-adbm, adalimumab-fjkg, adalimumab-ryvk), infliximab (generic for Remicade), Otezla, or Taltz; **OR**

- b. A contraindication to ALL of the following: Enbrel, adalimumab, infliximab (generic for Remicade), Otezla, and Taltz; **AND**

6. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A)

7. *If the 90 mg syringe is requested*, approve if the member meets ALL of the above and one of the following:

- a. Member weighs > 100 kg; **OR**
- b. Member is currently receiving the 90 mg syringe; **OR**
- c. Member has received standard dosing with the 45 mg syringe/vial for at least three months with inadequate efficacy.

Psoriatic Arthritis (PsA) :

For the approval of the 45 mg syringe/vial

- 1. The member is 6 years of age or older; **AND**
- 2. The requested medications prescribed by or in consultation with a dermatologist or rheumatologist; **AND**
- 3. Documentation of ONE of the following:
 - a. An inadequate response or adverse reaction to a three consecutive month trial of one non-biologic DMARD or contraindication to ALL non-biologic DMARDs or current use of a non-biologic DMARD; **OR**
 - b. An inadequate response or adverse reaction to at least a three consecutive month trial of one biologic DMARD that is FDA-approved for Psoriatic Arthritis (PsA); **AND**
- 4. Documentation of ONE of the following:
 - a. An inadequate response or adverse reaction at least a three consecutive month trial of Enbrel Humira or a preferred

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adalimumab biosimilar product (adalimumab-aacf, adalimumab-adaz, adalimumab-adbm, adalimumab-fjkg, adalimumab-ryvk),
Xeljanz infliximab (generic for Remicade), Otezla, Rinvoq, or Taltz;

OR

- b. A contraindication to ALL of the following: Enbrel, adalimumab, Xeljanz infliximab (generic for Remicade), Rinvoq, Otezla and Taltz; **AND**

- 5. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A); **AND**

- 6. *If the 90 mg syringe is requested*, approve if the member meets ALL of the above and ONE of the following:

- a. Member weighs > 100 kg; **OR**
- b. Member is currently receiving the 90 mg syringe; **OR**
- c. Member has received standard dosing with the 45 mg syringe/vial for at least three months with inadequate efficacy.

Moderate to severely active Ulcerative Colitis (UC):

- 1. The member is 18 years of age or older; **AND**
- 2. The requested medication is prescribed by or in consultation with a gastroenterologist; **AND**
- 3. Documentation of one of the following:
 - a. An inadequate response or adverse reaction to at least a three consecutive month trial of ONE of the following or a contraindication to them ALL:
 - i. 5-aminosalicylic acid, e.g. mesalamine (for induction and/or maintenance of remission)
 - ii. thiopurines, e.g. azathiopurine or mercaptopurine (for maintenance of remission only)
 - iii. corticosteroids(for induction of remission only); **OR**

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	b. An inadequate response, or adverse reaction to at least a three consecutive month trial of one biologic DMARD that is FDA-approved for UC; AND c. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A); AND d. Documentation of one of the following: a. An inadequate response or adverse reaction to at least a three month consecutive trial of Humira or a preferred adalimumab biosimilar product (adalimumab-aacf, adalimumab-adaz, adalimumab-adbm, adalimumab-fjkg, adalimumab-ryvk), Rinvoq, Xeljanz, or infliximab (generic for Remicade); OR b. A contraindication to ALL of the following: adalimumab, Rinvoq, Xeljanz, and infliximab (generic for Remicade).
Coverage Duration	Initial: 6 months Continuation: 12 months
Other criteria	Reauthorization: 1. Initial criteria have been met; AND 2. One of the following: a. The member has had a positive response to therapy; OR b. Documentation of clinical justification to continue therapy with the requested medication

Appendix A – FDA Approved Dosing

Diagnosis	FDA Approved Dosing
Crohn Disease	Induction: IV: ≤55 kg: 260 mg as single dose, >55 kg to 85 kg: 390 mg as single dose, >85 kg: 520 mg as single dose Maintenance: SUBQ: 90 mg every 8 weeks; begin maintenance dosing 8 weeks after the IV induction dose.
Plaque psoriasis	≤100 kg: 45 mg at 0 and 4 weeks, and then every 12 weeks thereafter.

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	>100 kg: 90 mg at 0 and 4 weeks, and then every 12 weeks thereafter.
Psoriatic arthritis	Initial and maintenance: 45 mg at 0 and 4 weeks, and then every 12 weeks thereafter. Coexistent psoriatic arthritis and moderate to severe plaque psoriasis in patients >100 kg: Initial and maintenance: 90 mg at 0 and 4 weeks, and then every 12 weeks thereafter.
Ulcerative colitis	Induction: IV: ≤55 kg: 260 mg as single dose, > 55 kg to 85 kg: 390 mg as single dose, >85 kg: 520 mg as single dose. Maintenance: SUBQ: 90 mg every 8 weeks; begin maintenance dosing 8 weeks after the IV induction dose.

Appendix B - Non-Biologic DMARD Treatment Options

Note: other trials may be considered on a case-by-case basis

Diagnosis	Non-Biologic DMARD Treatment Options
Plaque Psoriasis	Methotrexate Azathioprine Cyclosporine
Psoriatic Arthritis	Methotrexate Leflunomide Sulfasalazine Azathioprine

Applicable Coding

Code	Medication
J3358	Stelara (ustekinumab injection)

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Clinical Background Information and References

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3. Elmetts, C. Lim, H, Wu, J. et. al. Joint American Academy of Dermatology–National Psoriasis Foundation guidelines of care for the management and treatment of psoriasis with phototherapy. Journal of the American Association of Dermatology. 81(3):p775-804. Sept. 2019. Accessed July 2024.
4. Feuerstein, Joseph D., et al. "AGA Clinical Practice Guidelines on the Medical Management of Moderate to Severe Luminal and Perianal Fistulizing Crohn's Disease." Gastroenterology, vol. 160, no. 7, June 2021, pp. 2496-508. Accessed July 2024.
5. Feuerstein, J. Isaacs, K. Schneider, Y. et. al. AGA Clinical Practice Guidelines on the Management of Moderate to Severe Ulcerative Colitis. Clinical Practice Guideline. 158 (5);p 1450-1461, April 2020. Accessed July 2024.
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Policy History

Original Approval Date	Original Effective Date	Policy Owner	Approved by
12/1/2020	1/1/2021	Pharmacy Services	Pharmacy & Therapeutics (P&T) Committee, NH DHHS

Policy Revisions History			
Review Date	Summary of Revisions	Revision Effective Date	Approved by
12/1/2020	9.193 Stelara Policy retired, new policy created. Updated age expansion for PS; updated policy for trial and failure to align with NH state PDL requirements	1/1/2021	P&T Committee, NH DHHS
8/12/2021	Criteria updates for CD to align with ACG guidelines, removal of 5-ASA and corticosteroid trial requirements. Removal of criteria for severe form of disease to bypass trial requirements for both CD and UC. Addition of criteria to	12/1/2021	P&T Committee, NH DHHS

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	all indications to allow a previous trial of a biologic DMARD to bypass non-biologic DMARD criteria and 3 month trial requirements or contraindication to all required trial agents.		
8/11/2022	Annual Review: Added language regarding IV dosing for induction therapy only	9/1/2022	P&T Committee, NH DHHS
8/19/2022	Update to reflect new PDL preferred agents in trial and failure requirements. Addition of age expansion ≥ 6 years or older for PsA and addition of dosing restrictions for 90mg dose for PsA and PS.	9/1/2022	P&T Committee, NH DHHS
7/1/2023	Update to reflect new PDL preferred agents in trial and failure requirements.	7/1/2023	P&T Committee, NH DHHS
8/10/2023	P&T annual review: Update CD and UC baseline criteria to align with guidelines. Require documentation of the use of required trial/failure agents prior to approval. Update reauthorization criteria to standard. Addition of dosing criteria and appendix.	12/1/2023	P&T Committee, NH DHHS
5/1/2024	PDL Alignment: Minor Formatting Changes. In the revised policy, preferred adalimumab biosimilar products are included as alternatives for diagnoses of Crohn's Disease, Plaque Psoriasis, Psoriatic Arthritis, and Ulcerative Colitis.	9/1/2024	P&T Committee, NH DHHS
8/8/2024	P&T annual review: No recommended changes.		P&T Committee,

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9/1/2024	Policy updated to reflect Rinvoq and adalimumab-ryvk as preferred product on PDL.	3/1/2025	P&T Committee, NH DHHS
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Next Review Date

8/2025

Other Applicable Policies

Reference to Applicable Laws and Regulations, If Any

Disclaimer Information

Medical Policies are the Plan's guidelines for determining the medical necessity of certain services or supplies for purposes of determining coverage. These Policies may also describe when a service or supply is considered experimental or investigational, or cosmetic. In making coverage decisions, the Plan uses these guidelines and other Plan Policies, as well as the Member's benefit document, and when appropriate, coordinates with the Member's health care Providers to consider the individual Member's health care needs.

Plan Policies are developed in accordance with applicable state and federal laws and regulations, and accrediting organization standards (including NCQA). Medical Policies are also developed, as appropriate, with consideration of the medical necessity definitions in various Plan products, review of current literature, consultation with practicing Providers in the Plan's service area who are medical experts in the particular field, and adherence to FDA and other government agency policies. Applicable state or federal mandates, as well as the Member's benefit document, take precedence over these guidelines. Policies are reviewed and updated on an annual basis, or more frequently as needed. Treating providers are solely responsible for the medical advice and treatment of Members.

The use of this Policy is neither a guarantee of payment nor a final prediction of how a specific claim(s) will be adjudicated. Reimbursement is based on many factors, including member eligibility and benefits on the date of service; medical necessity; utilization management guidelines (when applicable); coordination of benefits; adherence with applicable Plan policies and procedures; clinical coding criteria; claim editing logic; and the applicable Plan – Provider agreement.

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